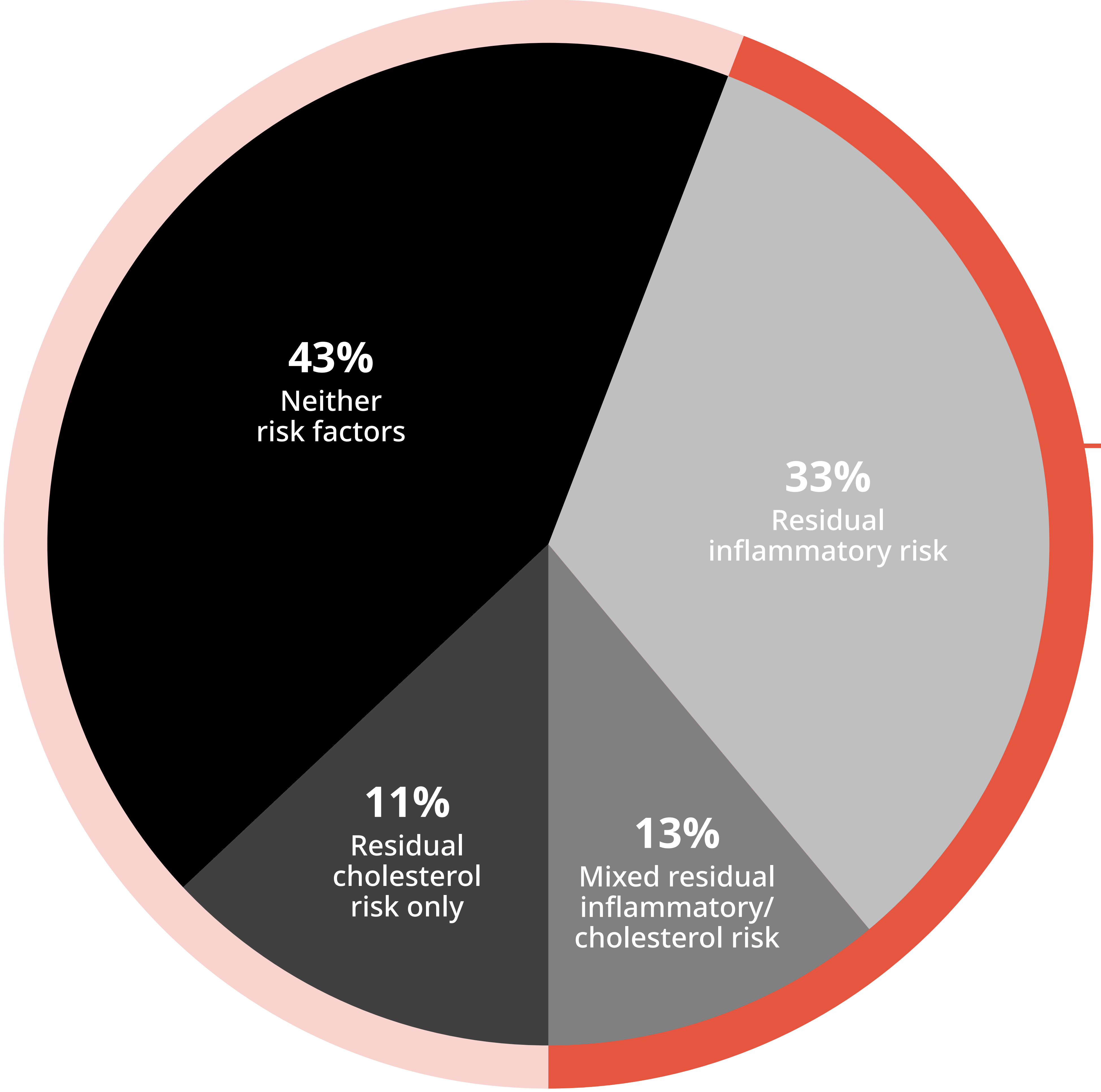
Even when protected by high-intensity lipid-lowering therapy, 1 in 2 CVD patients are still in danger of inflammatory risk[1]*

In RCTs, 46% of patients have a residual inflammatory risk despite lipid management with statins and PCSK9i[1]

Data across the PROVE-IT[2], IMPROVE-IT[3] and SPIRE-1/2[4], which included patients with/at high risk of CVD

Residual inflammatory risk = hsCRP ≥2 mg/L[1]
Residual cholesterol risk = LDL-C ≥70 mg/dL[1]



46%

Residual inflammatory risk

Residual inflammatory risk was identified in 46% of patients after high-intensity lipid-lowering therapy[1]*

CVD=cardiovascular disease; hsCRP=high-sensitivity C-reactive protein; LDL-C=low-density lipoprotein cholesterol; PCSK9i=proprotein convertase subtilisin/kexin type 9 inhibitor; RCT=randomised controlled trial.

1. Ridker PM. J Am Coll Cardiol 2018;72:3320–3333. **2.** Cannon CP et al. N Engl J Med 2024;350(15):1495–1504. **3.** Spinar J et al. Vnitr Lek 2014;60(12):1095–1101. **4.** Pradhan AD et al. Circulation 2018;138(2):141–149.

*As calculated by Ridker[1], across the PROVE-IT (n=4,162)[2], IMPROVE-IT (n=18,144)[3], and SPIRE-1/SPIRE-2 trials (n=9,738; trial data were combined for analysis due to design similarity)[4], patients with CVD received high-intensity statins, high-intensity statins plus ezetimibe, or high-intensity statins plus PCSK9 inhibition, respectively. Of these patients, 33% had inflammatory risk alone (hsCRP ≥2 mg/L), whereas 13% had a mix of inflammatory and cholesterol risk (LDL-C ≥70 mg/dL).[1]

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